



NMHS POLICY

Voluntary Assisted Dying

Scope (Staff)	All clinical and non-clinical staff
Scope (Area)	All NMHS sites and services
Summary	This Policy outlines North Metropolitan Health Service's approach to Voluntary Assisted Dying and provides staff with guidance in relation to their roles and responsibilities under the <i>Voluntary Assisted Dying Act 2019</i>. This Policy has been reviewed and updated to include the following revisions: • expansion and clarification of definitions; • clarification of information regarding use of language services; • clarification of Medical Practitioners role; • expansion and clarification of the patient referral pathways; • clarification or documentation requirements.
Approval	North Executive Team
Sponsor	Executive Director, Consumer Experience and Clinical Excellence
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1. Aim

This Policy outlines North Metropolitan Health Service's (NMHS) approach to Voluntary Assisted Dying (VAD) and provides staff with guidance in relation to their roles and responsibilities under the [Voluntary Assisted Dying Act 2019](#) (the Act). It supports the participation in VAD at NMHS in accordance with the Act, and the WA Department of Health Mandatory Policy on [Managing Voluntary Assisted Dying \(MP 0154/21\)](#).

This Policy is applicable to all NMHS clinical and non-clinical staff, including those with a conscientious objection to participation in the VAD process. This Policy should be read in conjunction with the Act, [WA Health Managing Voluntary Assisted Dying \(MP 0154/21\)](#) and the [Western Australian Voluntary Assisted Dying Guidelines](#).

This Policy is supported by the [NMHS Procedure: Voluntary Assisted Dying](#).

2. Risk

Non-compliance with this Policy may result in a breach of the legislative and mandatory requirements, harm to patients, staff or others, and/or reputation and image damage to NMHS. Contravention of a provision within the Act is capable of constituting professional misconduct or unprofessional conduct, which upon review may attract prosecution and/or penalty.

3. Policy Principles

- VAD is distinctly different from other end-of-life (EOL) care practices including palliative care, withholding and withdrawing life sustaining treatment, treating symptoms with medications in those nearing the EOL and terminal sedation.
- VAD is an elective medical procedure and not an emergency. While expressions of distress and suffering need urgent attention, all requests to access VAD should be acknowledged with careful, considered conversation and collaboration.

- **A patient has the legal right to make a First Request in relation to VAD, and have a First Assessment undertaken by a VAD practitioner regardless of perceived eligibility or ineligibility.**

- Only patients who are assessed as eligible as per the Act by a VAD practitioner can access VAD, noting that disability and mental illness alone do not satisfy the eligibility criteria.

- **All health professionals, regardless of any conscientious objection, must be aware of their responsibilities under the Act and must not actively obstruct patient access to information and/or advice regarding the VAD process.**

- All staff must respect the confidentiality of patients and staff engaged in the VAD process. Care should be taken to maintain a patient's right to confidentiality in respect to VAD. Specific offences (including terms of imprisonment) exist under the VAD Act for the non-permitted disclosure of information. Any discussions regarding VAD with the patient's family including next of kin, any health care professionals or health care service providers, or their General Practitioner (GP) must have the patient's consent.

- Consent from the patient must be obtained when making referrals to other practitioners, professionals or services in the VAD process (including the [WA VAD Statewide Care Navigator Service](#)). This must be documented in the patient's healthcare record.
- All staff must ensure the privacy of patients and their families is respected at all times during the VAD process including having discussions in relation to VAD in appropriate rooms and locations, wherever practicable.
- Each case of VAD is unique, with its own ethical, practical and emotional considerations and patients seeking access to VAD will be reviewed on a case-by-case basis.
- Communication throughout the VAD process should occur in a culturally aware and sensitive manner. An approved interpreter service may be used to support the patient through the VAD process.
- Planning for access to VAD will generally be undertaken during usual business hours. Administration of the VAD substance will be discussed and planned with the patient and the Coordinating/Administering Practitioner in collaboration with relevant stakeholders such as admitting wards and teams.

4. NMHS Approach to VAD

- NMHS staff are encouraged to engage in conversations with patients, where appropriate, about suffering, death and dying to better understand their end-of-life wishes which may include access to palliative care and VAD. Refer to [Section 6](#) below for information on who may initiate discussions relating to VAD and when.
- All staff must understand their legal rights and obligations in relation to the Act. However, participation by health professionals in the VAD process is voluntary. No provider will be disadvantaged for their choice of involvement or degree of involvement in the VAD process.
- Where possible, eligible patients will be encouraged to communicate their wishes with their GP and any other health practitioners involved in their care. Patients should be encouraged to plan for VAD in the community for optimal autonomy. However, personal circumstances, choices and wishes must be respected and facilitated with assistance from the [WA VAD Statewide Care Navigator Service \(SWCNS\)](#) and/or the [NMHS VAD Coordinator](#).
- VAD within NMHS will be supported and coordinated by the [NMHS VAD Coordinator](#). Any queries related to VAD, including First Requests, should be referred to the [NMHS VAD Coordinator](#) (refer to [NMHS VAD Procedure](#) for Roles and Responsibilities of NMHS VAD Coordinator).

5. VAD eLearning and Education

All NMHS sites and services must determine which staff groups are required to complete the VAD eLearning and education sessions. Refer to [NMHS Mandatory Training Policy](#).

6. Initiating a discussion about Voluntary Assisted Dying (VAD)

A patient may raise the topic of VAD with anyone involved in their care.

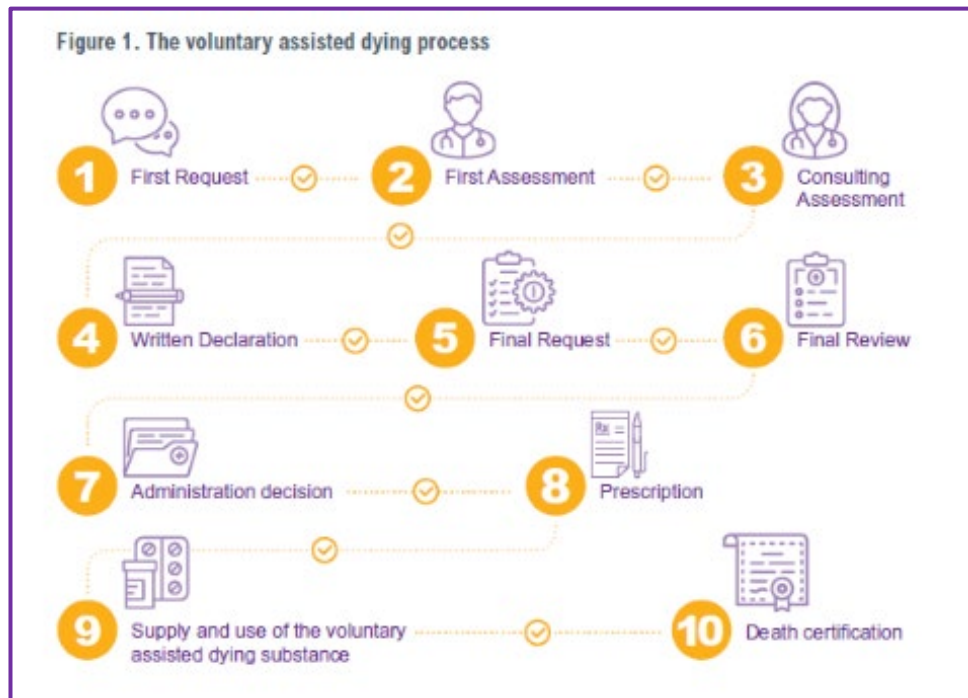
A medical practitioner or nurse practitioner can initiate a discussion about VAD with a patient if, at the same time, they also inform the patient of the palliative care and treatment options available to the person and the likely outcomes of that care and treatment.

Raising VAD outside of this constitutes unprofessional conduct within the meaning of the *Health Practitioner Regulation National Law (Western Australia) Act 2010*.

Interpreting assistance from a carer, family member or friend of the patient is **not** permitted under the Act. Interpreters (including Aboriginal Australian language interpreters) used during the VAD process must be accredited by a body approved by the Director General of Health for involvement in VAD. The [WA Health Language Services Guidelines](#) provides guidance supporting WA health staff to engage appropriate Language Services. Use of Language Services during the VAD process must adhere to the [WA Health Language Services Policy \(MP 0051/17\)](#). Refer to [NMHS VAD Procedure](#) Section 3 – Cultural Considerations for further details.

7. The VAD Process

There are ten (10) steps in the VAD process. Steps 1 to 6 involve the request and assessment process, while steps 7 to 10 relate to the administration process. This Policy covers Step 1 'First Request' in detail. Refer to [NMHS VAD Procedure](#) and the [Western Australian Voluntary Assisted Dying Guidelines](#) for more information including the assessments and the supply and management of the VAD substance (Step 2 - Step 10).



7.1 Responding to a First Request

Only medical practitioners can receive a First Request. If a patient makes a First Request to any other health professional, they should be informed that they must make their request to a medical practitioner and also be referred to the [NMHS VAD Coordinator](#) as soon as practicable to guide them with accessing an eligible and available medical practitioner.

Medical staff less than PGY10 are **not** eligible to fulfil a role under the Act and must **refuse** the First Request. They must complete steps 1) to 4) as described in **Box 1** below.

Box 1 Mandatory steps required under the Act

If a patient has made a First Request, the medical practitioner must complete below steps, regardless of whether accepting or refusing the First Request:

- 1) Accept or refuse the First Request and advise the patient of their decision;
- 2) Provide the patient with the [Approved information for a person making a First Request for voluntary assisted dying](#) (available from the NMHS Intranet VAD hub or the WA DOH VAD website);
- 3) Record in the patient's healthcare record the following information:
 - a First Request has been made;
 - the decision to accept or refuse;
 - if refused, reason for refusal; and
 - if they have provided the patient with the approved information;
- 4) Complete and submit the *First Request Form* to the VAD Board within two (2) business days of receiving the request via the VAD – Information Management System ([VAD-IMS](#)) or via fax. The form is available on the [VAD-IMS](#) website and log-in is not required.

Failure to complete any of these four (4) steps is a breach of the Act.

Submission to VAD-IMS does not trigger patient follow-up. The [NMHS VAD Coordinator](#) must be contacted for patient follow-up regardless of the decision.

If a medical practitioner refuses the First Request on the basis of conscientious objection, they must inform the patient **immediately** and complete the steps 1) to 4) as described above, and notify the [NMHS VAD Coordinator](#) as soon as practicable.

If a medical practitioner wishes to have time to consider the request, they must inform the patient of their decision within two (2) business days (not including the day the First Request was made), and complete the steps 1) to 4) as described above, and notify the [NMHS VAD Coordinator](#) as soon as practicable.

A medical record form (VAD – First Request Form) is available in NMHS sites to facilitate the necessary documentation of First Request in the patient's healthcare record. Refer to [NMHS VAD Procedure](#) Section 14 - Documentation for further information. Submission of an electronic First Request Form to the VAD Board is still required to be completed and uploaded electronically via [Voluntary Assisted Dying - Information Management System \(VAD-IMS\)](#).

7.2 Refer to [NMHS VAD Procedure](#) for information on Step 2 – Step 10 of the VAD process

8. Practitioner Roles

The Act allows medical practitioners and nurse practitioners who meet the eligibility criteria, and who have completed the approved training, to participate in the VAD process. It is important to note that under the Act, a medical or nurse practitioner must successfully complete the WA VAD Approved Training before engaging in actions required by the roles of **Coordinating Practitioner**, **Consulting Practitioner** or **Administering Practitioner**.

Refer to [WA VAD Guidelines, section 4.2](#) for eligibility criteria.

NMHS supports VAD practitioners, both internal and external to NMHS, seeking access to NMHS sites to support patients with the VAD process. All VAD practitioners (both internal and external to NMHS) must be credentialed to include VAD in their scope of clinical practice by the relevant Credentialing Committee. Practitioners external to NMHS will need to be credentialed on a case-by-case basis at the discretion of relevant Credentialing Committee before performing VAD related roles at NMHS sites.

The [NMHS VAD Coordinator](#) can be contacted to facilitate timely, out-of-session credentialing to avoid delaying the VAD process.

8.1 Coordinating and Consulting Practitioners

The Coordinating Practitioner may be a member of the patient's treating team, or an internal or external VAD practitioner. If a member of the patient's treating team cannot act as the Coordinating Practitioner for the patient, they may liaise with the [NMHS VAD Coordinator](#) or [SWCNS](#) to arrange a referral to a suitable VAD practitioner, with the patient's consent.

Where the Coordinating Practitioner or Consulting Practitioner is unable to determine the patient's eligibility to access VAD they **must** refer the patient to a registered medical practitioner or other professional with the appropriate skills and training to provide a determination in relation to the matter. The registered medical practitioner or professional who accepts the referral **must not** be a family member of the patient or be a beneficiary under the Will of the patient and **does not** need to have completed the WA VAD Approved Training.

8.2 Administering Practitioners

A medical practitioner is eligible to act as an Administering Practitioner if they meet the eligibility requirements to be a Coordinating Practitioner or Consulting Practitioner and they:

- are not a family member of the patient;
- are not a beneficiary under a will of the patient or may otherwise benefit financially or in any other material way from the death of the patient, other than by receiving reasonable fees for the provision of services;
- have successfully completed the WA VAD Approved Training.

A Nurse Practitioner is eligible to act as an Administering Practitioner if they:

- hold nursing registration with endorsement as a Nurse Practitioner, have practised as a nurse practitioner for at least two years and meet the requirements approved by the Director General of Health (as the CEO);

- are not a family member of the patient;
- are not a beneficiary under a will of the patient or may otherwise benefit financially or in any other material way from the death of the patient, other than by receiving reasonable fees for the provision of services;
- have successfully completed the WA VAD Approved Training.

9. Non-Practitioner Roles

9.1 Witnesses

9.1.1 Witnesses to Written Declaration

If the patient has been assessed as eligible for VAD by both the Coordinating Practitioner and the Consulting Practitioner, the patient must then make a **Written Declaration** in the presence of two (2) eligible witnesses. To be eligible to witness the signing of the Written Declaration, the two witnesses must:

- have reached 18 years of age;
- not be a beneficiary under the will of the patient or may otherwise benefit financially or in any other material way from the death of the patient;
- not be a family member of the person;
- not be the Coordinating Practitioner or Consulting Practitioner for the patient.

NMHS recommends that NMHS staff do **not** act as witnesses for a Written Declaration, however, NMHS staff *may* act as witnesses should they choose to do so. In addition to meeting the above requirements to be a witness, the following requirements must also be met:

- asked by the patient to be a witness to the Written Declaration; and
- comfortable and willing to do so.

Refer to [NMHS VAD Procedure](#) Section 5.4 – Written Declaration for detailed information.

9.1.2 Witnesses to Practitioner Administration of the VAD substance

When planning for practitioner administration, the Administering Practitioner should discuss with the patient the requirement for an eligible witness to be present at the time of administration. It is important that the patient is comfortable with the witness and that the witness is prepared to take on the role. The patient may have selected several carers, family or friends to be with them at the end of their life. If willing, one of these people may act as the witness to the practitioner administration of the VAD substance if the administering practitioner is agreeable.

NMHS recommends that NMHS staff do **not** act as witnesses for Practitioner Administration, however, NMHS staff *may* act as witnesses if they choose to do so.

An eligible witness to administration:

- must have reached 18 years of age;
- must not be an ineligible witness (i.e. they must not be a family member of the Administering Practitioner and must not be employed or engaged under a contract for services by the Administering Practitioner).
- Refer to [NMHS VAD Procedure](#) Section 9.4.2 - Practitioner Administration for detailed information.

9.2 Contact Person

If a patient makes a self-administration decision (refer to [WA VAD Guideline, section 13.2.1](#)), they must appoint a Contact Person to comply with the Act. The Contact Person is permitted to:

- receive the VAD substance from the authorised supplier (the Statewide Pharmacy Service);
- possess the VAD substance for the purpose of supplying the substance to the patient;
- supply the VAD substance to the patient;
- possess the VAD substance for the purpose of giving the substance, or any unused or remaining portion of the substance, to an Authorised Disposer.

NMHS staff **should not** be a Contact Person for a patient accessing VAD unless the patient is a family member/relative, next of kin or close friend. In exceptional circumstances where all other alternatives are exhausted, NMHS staff *may* act as a Contact Person for inpatient administration. In both situations, the staff must have a full understanding of the role and responsibilities of a Contact Person under the Act before agreeing to take on the role (Refer to [DOH Being the Contact Person information sheet](#)). The NMHS VAD Coordinator can be contacted to provide further guidance if required.

10. Patient Referral Pathways

Referrals for components of the VAD assessment and/or elective admissions for the purposes of VAD Administration at NMHS may come from the following sources:

- Medical Practitioners from inpatient treating teams
- The SWCNS;
- Outpatient clinics at NMHS;
- Faith-based organisations within the NMHS catchment who do not participate in the VAD process; or
- GPs within the NMHS catchment.

Any patient seeking an elective admission for the purposes of VAD Administration at NMHS will be admitted under the care of either their known treating consultant, another specialty consultant, or under the Director of Clinical Services (or their delegate) once acceptance of care has been confirmed by the NMHS VAD Coordinator.

The elective admission process will be coordinated by the [NMHS VAD Coordinator](#), in liaison with site executives.

11. Documentation and Patient Records

Health professionals must ensure they document any conversations with the patient, their family or carer in relation to VAD in the patient's healthcare record. As a guide, the following should be documented:

- the context of the conversation and who has initiated the discussion;
- the exact phrases used by the patient, where possible;
- the exact phrases of the health professional's response, where possible, and
- a summary of the conversation, actions of the health professional including any escalation responses or referrals.

The VAD – First Request medical record form is available at all NMHS sites and via the [NMHS VAD Coordinator](#) to facilitate the documentation of a First Request in the patient's healthcare record as describes in [Section 7.1](#).

For patients who have not accessed NMHS previously and are admitted for administration of the VAD substance, a healthcare record must be created. There are minimum documentation requirements, which are similar to the standard required for a same day admission.

For detailed documentation requirements in relation to VAD, refer to [NMHS VAD Procedure](#) Section 14 - Documentation.

Entries in uncontrolled documents (eg handover sheets, discharge summaries) should not mention VAD unless specific patient consent has been sought. Doing so could place a practitioner at risk of breaching s106 of the Act. A generally acceptable alternative is 'end-of-life care'.

12. The legal context of VAD for practitioners

12.1 Protections, offences and professional misconduct or unprofessional conduct

The Act provides protections for staff involved in the VAD process but also clearly articulates offences and circumstances that may be considered professional misconduct or unprofessional conduct.

12.1.1 Protections

The Act provides protection from unprofessional conduct, civil and criminal liability for a person, including NMHS staff, who:

- assist a patient to request access to VAD;
- assist a patient to access VAD;
- is present when the VAD substance is administered;
- in good faith, and with reasonable care and skill, does a thing in accordance with the Act or believes the thing is done in accordance with the Act.

This protects staff who do not administer lifesaving treatment in circumstances where the patient does not request it and the staff member believed, on reasonable grounds, that the patient is dying after self-administering or being administered the VAD substance in accordance with the Act. Protection is applicable to registered health practitioners, ambulance officers (and volunteers) and other people in roles which usually imply a duty to administer lifesaving treatment.

12.1.2 Offences

Offences in relation to VAD are listed below:

- unauthorised administration of the VAD substance;
- inducing a patient to request or access VAD;
- inducing self-administration of the VAD substance;
- providing false or misleading information for any purpose under the Act;
- advertising Schedule 4 or Schedule 8 poison as a VAD substance;

- recording, use or disclosure of information obtained under the Act;
- publication of patient information concerning a proceeding before the SAT under the Act; and
- failure to give a prescribed form under the Act to the VAD Board.

Penalties for committing VAD related offences include monetary fines and imprisonment.

12.1.3 Professional Misconduct or Unprofessional Conduct

A breach of a provision of the Act by a registered health practitioner may be professional misconduct or unprofessional conduct for the purposes of *the Health Practitioner Regulation National Law (Western Australia) Act 2010*. This is the case even if the breach is not an offence under the Act. The VAD Board can refer matters to both the Australian Health Practitioner Regulation Agency (Ahpra) and the Health and Disability Services Complaints Office (HaDSCO).

12.2 The Commonwealth *Criminal Code Act 1995*

The Commonwealth *Criminal Code Act 1995* contains offences which limit the use of a carriage service to access and transmit suicide-related material. This directly influences how particular parts of the voluntary assisted dying process can be communicated.

As a general rule, any information that relates specifically to the act of administering a VAD substance or provides details or instructions about the act of administering a VAD substance must not be discussed or shared by phone, fax, email, videoconference, internet and the like.

Informing people about the legislation and associated processes in WA (either generic or in relation to a person's circumstance) may be undertaken via a carriage service to the extent that the information **does not advocate, encourage, incite, promote or teach about how to undertake the act of administration of a VAD substance.**

13. Support Resources

NMHS acknowledges that staff may be personally affected by VAD in different ways. The [Employee Assistance Program \(EAP\)](#) can be accessed for support. Additional support can be provided by the [NMHS VAD Coordinator](#) or the [SWCNS](#). Refer to the [NMHS VAD Procedure Section 15 - Support Resources](#) for more information.

14. Managing Complaints Relating to VAD

Consumer feedback in relation to VAD should be managed in accordance with usual complaint management process.

Staff are encouraged to provide feedback on the NMHS VAD process directly to the [NMHS VAD Coordinator](#). Feedback can also be directed to the VAD Board via a [Personal Reflections form](#). Feedback about the process for practitioners accessing the WA VAD Approved Training can be provided via email to the End of Life Care Program at the Department of Health (VADApprovedTraining@health.wa.gov.au).

15. Definitions

Administering practitioner	The medical practitioner or nurse practitioner who administers the VAD substance to a patient. If the patient chooses practitioner administration, the Coordinating Practitioner can be the Administering Practitioner.
Carriage Service	Communication aids including phone, fax, email, videoconferencing (including Telehealth), internet and the like.
Conscientious Objector	A person who has considered VAD and whose beliefs and values conflict with VAD and do not wish to participate in any aspect of VAD process.
Conscientious Provider	A person who has considered how VAD aligns with their beliefs and values and is willing to participate in the process. The level of participation may vary from person to person and may change and vary with time.
Consulting Practitioner	A medical practitioner who accepts a referral to conduct a consulting assessment of the patient.
Contact Person	A person appointed by a patient (under section 65(1) of the Act) who has made a self-administration decision to carry out specific activities required by the Act. Contact Person has the responsibility of returning any unused or remaining VAD substance to the WA VAD Statewide Pharmacy Service. Other specific roles are explained further in the Being the Contact Person information sheet .
Coordinating Practitioner	The medical practitioner who accepts the patient's First Request or the Consulting Practitioner who accepts the transfer of the role of the Coordinating Practitioner.
Eligible medical practitioner	A medical practitioner who meets the eligibility criteria listed under section 17 of the Act and who has successfully completed the WA VAD Approved Training. Eligibility criteria can be found in the WA VAD Guidelines, section 4.2
Eligible nurse practitioner	A nurse practitioner who meets the eligibility criteria listed under section 54 of the Act and who has successfully completed the WA VAD Approved Training. Eligibility criteria can be found in the WA VAD Guidelines, section 4.2
First Request	A clear and unambiguous request for access to VAD made to a medical practitioner during a medical consultation under section 18(1) of the Act. This is the first of three requests a patient must make as part of the process to access VAD.
Final Request	The final request for access VAD that a patient makes to the Coordinating Practitioner after completing the Written Declaration. This is the last of three requests a patient must make to access VAD.
Medical Consultation	An appointment or meeting between a medical practitioner and a patient to seek medical advice or treatment.
Nurse	In this document, the term 'nurse' encompasses registered nurses and midwives.
Nurse Practitioner	A person registered in the nursing profession (other than as a student) whose registration is endorsed as nurse practitioner. A nurse practitioner has an additional masters degree and is licensed to work at an advanced practice level.

Palliative Care	An approach to care that improves the quality of life of people and their families who are facing the problems associated with a progressive illness. It aims to neither prolong nor hasten death. The aim of Palliative care is to prevent and relieve suffering through early identification and assessment, treat pain and other physical, psychosocial, and spiritual distress and address practical issues resulting from progressive or life-limiting illness.
Self administration	The process whereby a person, who has been assessed as eligible for VAD and made a self-administration decision, prepares and ingests the VAD substance themselves.
Statewide Care Navigator Service (SWCNS)	The service established to provide a statewide point of contact for information and assistance relating to VAD in Western Australia. This includes help locating a practitioner who is willing and eligible to participate and providing support to staff and patients.
Statewide Pharmacy Service (SWPS)	The statewide pharmacy service established to ensure the VAD substance is provided in a safe, equitable, and patient-centred manner.
Voluntary Assisted Dying (VAD)	The legal process that enables an eligible person to access, administer or have administered to them, the VAD substance for the purpose of causing their death.
VAD Practitioner	Medical Practitioners and Nurse Practitioners who meet the eligibility requirements, and who have successfully completed the WA VAD Approved Training to participate in the voluntary assisted dying process as a VAD Practitioner
VAD substance	A controlled substance for the purpose of causing a person's death, referred to as the 'VAD substance' in the Act.
Written Declaration	The Written Declaration is a formal written request for access to VAD. This is the second of three requests a person must make as part of the process to access VAD.

16. Compliance and Evaluation

Each NMHS site or service Executive Director is to ensure compliance with this policy.

There are legislative obligations relating to this Policy for which a statutory penalty is imposed for a breach. Refer to the [NMHS VAD Procedure](#) for more information about legislative obligations.

17. Document Control

Version	Amendment(s)	TRIM Reference	Published Date
1.0	Original	D/21/53194	28/06/2021
1.1		D/21/52992	30/06/2021
2.0	This Policy has been reviewed as scheduled with amendment made: • Expansion of definitions; • Inclusion of information regarding role of NMHS VAD Coordinator; • Clarification of documentation requirements.	D/21/78269	17/01/2022

3.0	This Policy has been reviewed as scheduled with major amendment made: • expansion and clarification of definitions; • clarification of information regarding use of language services; • clarification of Medical Practitioners role; • expansion and clarification of the patient referral pathways; • clarification or documentation requirements. A new policy cycle will commence.	D/23/141981	30/10/2023 (NET approved 20/10/2023)
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Related policies, procedures and guidelines

WA Health Policy Framework/Mandatory Policies

[Credentialing and Defining Scope of Clinical Practice Policy \(MP 0084/18\)](#)

[High Risk Medication Policy \(MP 0131/20\)](#)

[Language Services Policy \(MP 0051/17\) and related documents](#)

[Language Services Guidelines](#)

[Managing Voluntary Assisted Dying Policy \(MP 0154/21\)](#)

[Medicines Handling Policy \(MP 139/20\)](#)

[Western Australian Voluntary Assisted Dying Guidelines](#)

NMHS Policy Documents

[NMHS Carers Policy](#)

[NMHS Clinical Documentation Policy](#)

[NMHS Healthcare Record – Access, Use and Disclosure Policy](#)

[NMHS Healthcare Record Management Policy](#)

[NMHS Management of Psychosocial Hazards in the Workplace Guideline](#)

[NMHS Mandatory Training Policy](#)

[NMHS My Health Record \(MHR\) System Control, Administration, Access, Upload and Security Procedure](#)

[NMHS Voluntary Assisted Dying Procedure](#)

[NMHS Witnessing of Patient Wills and Other Legal Documents Policy](#)

References

[Coroners Act 1996](#)

[Criminal Code Act 1995 \(Commonwealth\)](#)

[Health and Disability Services \(Complaints\) Act 1995](#)

[Health Practitioner Regulation National Law \(Western Australia\) Act 2010](#)

[Medicines and Poisons Act 2014](#)

[Mental Health Act 2014](#)

[Voluntary Assisted Dying Act 2019](#)

Resources

[Employee Assistance Program \(EAP\)](#)

[Voluntary Assisted Dying - Information Management System \(VAD-IMS\)](#)

[WA Health Voluntary Assisted Dying resources for health professionals](#)

Acknowledgement

Some content in this Policy is based on the resources developed by the East Metropolitan Health Service and the Austin Health (Melbourne). This Policy is also informed by the *Voluntary Assistant Dying Act 2019 (WA)* and the Managing Voluntary Assisted Dying Policy (MP 0154/21) and the Western Australian Voluntary Assisted Dying Guidelines developed by the Western Australia Department of Health.

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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